



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : اميره عبد المنعم امين عبد المنعم

العنوان : ----

المحافظة : الجيزة

الحى / المدينة : العجوزه

تاريخ تسجيل الإستمارة : 10-10-2026

رقم البطاقة : 28207240104741 رقم التليفون : 01003106682

العمر (تاريخ الميلاد) : 24-07-1982 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي الفقرات القطنيه بالصبغه

قسم العلاج :
الأورام

[12771]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة :

الوظيفة :

الوظيفة :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة

رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

قسم :

جهة العلاج :

نفقات :

ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: اميره عبد المنعم امين عبد المنعم National ID: 28207240104741	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced MRI of the lumbar spine
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Name : Amira abd el moneem anteen
Branch : Portable
Ref.Physician Prof : et cap+c

Date : 10-02-2026 01:18
Medical No. : GRI-54825
Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA. no available previous films for comparison.

I- CHEST:

- Small right middle lobe pulmonary nodule measuring 4 mm.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- Few subcentimetric paracardiac lymph nodes.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Few subcentimetric para-aortic and bilateral external iliac lymph nodes.
- No ascites.

Opinion:

❖ Small right middle lobe pulmonary nodule.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Name: اميرة عبد المنعم امين		GUSTAVE ROUSSY	
MR. : 1220			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
		بقسم:	
رقم طبي:			
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها			
The patient was initially diagnosed with de novo metastatic breast cancer to the lung. She underwent upfront surgery prior to the discovery of pulmonary metastases and subsequently received 11 weeks of taxane (taxol) plus intravenous trastuzumab (herceptin), which was discontinued because of severe neuropathy. She is now on endocrine therapy (ET) combined with intravenous trastuzumab; the current trastuzumab cycle is number 21, and a dose of zoladex was due on 3 March 2025. The patient has a history of left-sided breast cancer treated with breast-conserving surgery (BCS) in April 2024. Post-operative imaging revealed pulmonary nodules. She received adjuvant taxol and trastuzumab, followed by hormonal therapy with letrozole (Femara) and a gonadotropin-releasing hormone agonist (Zoladex) initiated in October 2024 for trastuzumab 440 mg and Femara. Zoladex was scheduled for administration on 3 March 2025 in conjunction with trastuzumab intravenous therapy pending new echocardiogram and computed tomography (CT) assessments. The patient underwent BCS in April 2024 before the detection of lung metastasis and was subsequently placed on letrozole (Femara) and intravenous trastuzumab. Small subcentimetric pulmonary nodules were noted. Zoladex was due on 3 March 2025 while awaiting approval; an echocardiogram performed on 20 January showed a left ventricular ejection fraction of 60 %. The patient was on trastuzumab cycle 24 with letrozole and Zoladex, and on the day of the note she was receiving trastuzumab cycle 25 and preparing for cycles 26, with pharmacy providing trastuzumab, letrozole, supportive therapy, and Zoladex. A CT on 10 February 2026 demonstrated a small right middle-lobe pulmonary nodule measuring 4 mm. Pharmacy records on 26 March 2026 indicated the patient's weight at 80 kg. An echocardiogram on 20 January showed an ejection fraction of 60 %. The patient received trastuzumab 440 mg (cycle 25), letrozole, and Zoladex 10.8 mg, along with supportive treatment. The patient's premenopausal left-breast cancer was initially staged as T2 N1 (11/11) with estrogen-receptor (ER) 8/8, progesterone-receptor (PR) 8/8, and HER2-positive disease. After BCS in April 2024, she received adjuvant taxol and trastuzumab, followed by Zoladex, letrozole, and continued trastuzumab. A CT in June 2022 showed bilateral pulmonary nodules, the largest measuring 9 mm; a subsequent CT in February 2026 demonstrated a 5 mm nodule. A repeat echocardiogram in October 2025 was pending for planning trastuzumab cycle 26 (440 mg) with letrozole and Zoladex, with the next Zoladex dose due on 18 June. On 3 June 2026 the patient presented for trastuzumab cycle 27. She was in good general condition and continued supportive therapy. On 5 July 2026 the patient received trastuzumab cycle 28 (440 mg) together with letrozole and Zoladex, with the next Zoladex dose scheduled for 22			
تحريرا في ,			
		التاريخ : 18-09-2026	
		الوقت :	
		dina makram	
		الطبيب المعالج :	
يعتمد ,			

June; the echocardiogram showed an ejection fraction of 65 %.

On 26 July 2026 the patient has metastatic breast cancer with subcentimetric pulmonary nodules. A CT chest/abdomen/pelvis performed in July 2026 showed complete resolution of the previously noted small right middle-lobe pulmonary nodule, indicating no radiographic evidence of metastasis. She received intravenous trastuzumab in the day-care pharmacy on the day of the visit.

Patient Name: Amira Abd Al Moneim Ameen Abd Al Moneim	Date : 29/12/2024
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 1488/2024	Pat. ID: 54825

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

III- The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Mild regression of the left breast lesion and associated underlying muscle thickening.

Otherwise; Stationary course of the rest of the study.

Dr. Shimaa Hussien Ali

Dr. Walid Mohamed, M.D.

Patient Name : الأستاذ / اميره عبدالمنعم امين عبدالمنعم
Patient No. : 28207240104741
Path No. : SR-637/24
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13009467
Request Date : 19/05/2024
Verification Date : 12/06/2024
Financial Category : دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hega

Pathology Report

Clinical Data: Left breast mass (WLE & Axillary clearance).
Nature of Specimen: Slide Revision

Gross Examination:

- 10 paraffin blocks labeled by (2409635) and 13 slides were received.

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- DCIS: No.
- Angioinvasion: No.
- Perineural invasion: No.
- Lymph node involvement: Positive one metastatic lymph node (1/11).
- IPT:
- ER: Positive nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Positive nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 40% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II.
Free recieved margins.
Positive one metastatic lymph node (1/11).
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).

Verified by :

Dr: Sagda El-Kady

Printing Date : 13/04/2025 10:26:00

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Printing By : .marwa moham

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الاستاذ / اميره عبدالمنعم امين عبدالمنعم
Patient No. : 28207240104741
Path No. : SR-637/24
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13009467
Request Date : 19/05/2024
Verification Date : 12/06/2024
Financial Category : - نعمة الدونة
Department : Out Patient
Registered by : Mrs. Mayada

Pathology Report

- Ki67: Positive in 40% of tumor cells

Signature
Dr. Marwa Mahmoud.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

Printing Date : 12/06/2024 10:51:11

Page 2 of 2

Printing By : marwa



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mizr El Qadema - Cairo, E



Patient Name: Amira Abd Al Moneim Ameen Abd Al Moneim	Date : 29/12/2024
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 1488/2024	Pat. ID: 54825

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

- ✚ **Reported case of left breast cancer with CBS.**
- ✚ **Compared to last study dated 21-9-2024, the current study revealed:**

I -Chest:

- **Regressive course regarding the:**
 - Left breast heterogeneous iso-intermediate parenchymal density and the thickening of the overlying skin.
 - Previously seen irregular ill-defined heterogeneous density seen at the lateral aspect of left pectoral muscles showing heterogeneous soft tissue density with surrounding fibrotic bands.
- **Stationary course** The previously noted few bilateral mostly subpleural small and tiny pulmonary nodules with the largest seen at lateral segment of the middle lung lobe still measuring about 0.6. No newly developed nodules.
- **Stationary course** regarding the previously noted prominent subcentimeteric axillary and supraclavicular lymph nodes.
- No newly developed lesions.
- No pleural or pericardial effusion.
- Patent trachea and major bronchi.
- Normal non-contrast CT appearance of the heart and major thoracic aorta.

II-Abdomen and pelvis:

- The liver is average in size with homogenous low CT parenchymal attenuation and regular outline. No definite hepatic focal lesions,
- No intrahepatic or extrahepatic biliary dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological enlarged abdominal or pelvic lymph nodes.

Psychiatric			
SURGERY			
MEDICAL ONCOLOGY			
SIGNATURE			
RADIOLOGY			
PATHOLOGY			
Nutrition			

to continue on Herceptin and ET (AI+ zoledex)
 CTS: Regression
 Pt finished 11 weeks of taxol + 3 cycles Herceptin However developed PN GR III.

MDT OUTCOME & PLAN

Pt on femara+herceptin
 pt underwent lt. BCS (4-2024) before discovery of lung metastasis
 denovo metastatic breast cancer to lung (CT chest: 12-6-2024)

Sonomammography

T	N	M	1 lung
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6X	ER	PR	Her 2 Neu	SISH	SISH COMBAT
	8/8	8/8	3		

Breast Imaging : Clinical data

Co morbidity	Menopausal status	Initial STAGE	GROUP	REASON OF M
marital status	RBS Score	BMI		
Family history	Phone			

2-10 number: Patient Age : 42 Y

A-Patient ID: 1- Name : 2- Date of birth : 3- Medical number: 83206398
 4- Sex: Female 5- Old Med : 1488-2024

MDT Form